

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 68 yrs | Sex: Male | MRN: SMC-04471932

Encounter Date: September 30, 2013 | Provider: Anita Desai, MD — Radiation Oncology | Doc ID: NCI-2013-0930-CONS

Multidisciplinary Oncology Consultation

Newly Diagnosed Prostate Adenocarcinoma — Staging & Treatment Planning

History

68-year-old man with biopsy-proven prostate adenocarcinoma, Gleason 3+4=7 (Grade Group 2), 5/12 cores. PSA 8.1 ng/mL. Significant cardiac history (LAD DES 2012), diabetes, prior heavy tobacco use (quit 2012). Seen jointly by Radiation Oncology (Dr. Desai) and Medical Oncology (Dr. Liu).

Staging Investigations — Results

Multiparametric prostate MRI (09/20/2013): 1.4 cm PI-RADS 4 lesion right mid-peripheral zone; organ-confined; no definite extracapsular extension; seminal vesicles uninvolved. Clinical stage cT2c.

Technetium-99m bone scan (09/24/2013): No scintigraphic evidence of osseous metastatic disease.

CT abdomen/pelvis (09/24/2013): No pelvic or retroperitoneal lymphadenopathy. No visceral metastasis.

Risk Stratification

NCCN intermediate-risk (unfavourable) prostate cancer: Grade Group 2, PSA 8.1, cT2c, >50% positive cores. Localized disease.

Treatment Discussion

Options reviewed: radical prostatectomy vs definitive external-beam radiotherapy (EBRT) ± short-course androgen deprivation therapy (ADT) vs active surveillance (not favored given unfavourable intermediate risk). Given cardiac comorbidity and surgical risk, patient elects definitive EBRT with short-course ADT.

- Plan**
- Begin ADT: leuprolide acetate 22.5 mg IM depot every 3 months for 18 months; first dose today. Add bicalutamide 50 mg PO daily x 4 weeks to prevent testosterone flare.
 - EBRT to prostate and seminal vesicles, 78 Gy in 39 fractions (IMRT) — planning CT and simulation to be scheduled (start anticipated November 2013).
 - Baseline testosterone and PSA drawn. Bone health: calcium 1200 mg + vitamin D 1000 IU daily; baseline DEXA.
 - Cardiology clearance obtained — ADT cardiovascular risk discussed.
 - Coordinate glucose monitoring (ADT may worsen glycemic control).

Medication	Dose	Route	Frequency	Indication
Leuprolide acetate depot	22.5 mg	IM	Every 3 months	ADT — prostate cancer
Bicalutamide	50 mg	PO	Daily x 4 weeks	Anti-flare
Calcium carbonate	1200 mg	PO	Daily	Bone protection on ADT
Cholecalciferol (Vit D3)	1000 IU	PO	Daily	Bone protection